



Medicare Current Beneficiary Survey (MCBS)

Request for Information Reference No.: HHS-CMS-RFI-2026-270043

NAICS Code: 541720 – Research and Development in the Social Sciences and Humanities

A. General Information

THIS IS A REQUEST FOR INFORMATION (RFI). This is NOT a solicitation for proposals, proposal abstracts, or quotations and in no way obligates the Government to award a task order.

The purpose of this RFI is to obtain knowledge and information for project planning purposes and for exploring alternative solutions and determining industry standards for the administration and operation of the Medicare Current Beneficiary Survey (MCBS) requirement.

B. Project Background

CMS is the largest single payer of health care in the United States, administering health insurance coverage for more than 120 million people across the Medicare, Medicaid, CHIP, and Exchange populations. The agency serves as an effective steward of taxpayer resources and a trustworthy partner in improving healthcare quality, accessibility, and affordability while safeguarding program integrity and preventing fraud, waste, and abuse. Central to this mission is putting patients first in the delivery of their healthcare needs. CMS continuously strives to advance these objectives in response to evolving care delivery approaches, technological advancements, and new statutory requirements.

To support continuous improvement and innovation, CMS relies on comprehensive data generated through administering the Medicare program. Enrollment, claims, and assessment data are essential in informing program evaluation and future program planning. However, while these data are an important resource, certain key activities – such as developing fiscal projections, evaluating how Medicare policies affect beneficiaries' health and healthcare experiences, planning innovative payment and service delivery models, and conducting strategic program planning – often require data that cannot be obtained from administrative sources alone. Essential information on beneficiary income, supplemental insurance coverage, non-Medicare payments (including out-of-pocket spending), and non-covered services can only be obtained by surveying beneficiaries directly. Likewise, data on beneficiary knowledge of the Medicare program, health history, access to care, and satisfaction with services must be collected directly from beneficiaries.

The MCBS is a continuous, multi-purpose longitudinal survey, representing the population of Medicare beneficiaries residing in the United States. Since 1991, the MCBS has met CMS' need for data on beneficiaries' unique perspectives and experiences by collecting comprehensive information on beneficiaries' social and medical risk factors and their relationship to healthcare utilization and health outcomes. Linking survey data with Medicare claims and assessment data, the MCBS provides critical insights for policy development, program evaluation, and understanding beneficiary experiences that cannot be obtained from either administrative or survey data alone. The MCBS is also the only source that provides complete information on total healthcare expenditures and sources of payment for

Medicare beneficiaries, including out-of-pocket costs, supplemental insurance payments, and spending on non-covered services.

The MCBS is designed to represent the full diversity of the Medicare population. The survey collects data directly from beneficiaries across all segments of Medicare, including those:

- enrolled in Fee-for-Service and Medicare Advantage
- aged 65 and over, as well as those aged 64 and below with certain disabling conditions
- who use healthcare services and those who do not
- living in community and facility settings

This comprehensive approach ensures that MCBS data reflect the experiences of all Medicare beneficiaries, not just those who are most visible in claims data or who interact frequently with the healthcare system.

Data from the MCBS are used extensively across the Federal government and research community to inform evidence-based policymaking and regulatory analysis. MCBS data are used to inform CMS and Congressional Budget Office projections of Medicare spending and to support Medicare Payment Advisory Commission's analyses of Medicare payment policy and spending trends. Beyond federal applications, the survey provides data to hundreds of external researchers annually. Findings from MCBS analyses are regularly published in peer-reviewed journals.

The breadth and depth of MCBS data support a wide range of critical needs across CMS, informing development of policies on emerging priority topics, design of new Innovation Center models, and validation of algorithms developed using administrative data. Data from the MCBS have been instrumental in informing advancements to the Medicare program, including the creation of benefits such as Medicare's Part D prescription drug benefit. The survey's longitudinal design and consistent data collection methodology enable stakeholders to respond to emerging policy questions, conduct rigorous program evaluation, and track changes in beneficiary experiences and outcomes as the Medicare program evolves.

The MCBS continuously evolves to meet CMS's changing data needs and policy priorities while maintaining core content areas that enable longitudinal analysis and trend monitoring. As the healthcare landscape shifts and new policy questions emerge, the survey adapts by thoughtfully retiring outdated content and introducing new measures that reflect current priorities. This commitment to relevance is evident in recent and ongoing enhancements to the survey.

In 2026, the MCBS received Office of Management and Budget (OMB) approval to redesign its sampling approach, which, once fully implemented, will enable the survey to produce robust state-level estimates and capture geography-specific healthcare experiences. Additionally, CMS requires the MCBS to have the flexibility to rapidly respond to emerging policy questions and urgent information needs. The survey must be capable of quickly fielding targeted questions to current respondents, providing timely insights on critical topics such as public health emergencies, implementation of new programs or benefits, or other time-sensitive policy issues.

The survey also continuously refines its methodology, incorporating advances in data collection techniques and identifying new approaches for delivering timely, policy-relevant data to stakeholders. CMS is committed to leveraging emerging technologies and innovative approaches to ensure the security of beneficiaries' protected information; enhance data quality and analytical capabilities; improve operational efficiency and reduce respondent burden; and make MCBS data more discoverable and accessible to diverse users. These enhancements reflect CMS' commitment to ensuring that MCBS remains a dynamic, data collection instrument capable of responding to the evolving needs of a policy-driven agency.

C. Survey Methodology and Operations

The MCBS currently employs a complex multistage probability sample design. At the first stage of selection, the sample consists of 104 primary sampling units (PSU) defined to be metropolitan areas and clusters of nonmetropolitan counties. At the second stage, a sample of 703 secondary sampling units consisting of Census tracts or groups of tracts (SSUs) is selected within the sampled PSUs. At the third and final stage of selection, stratified samples of beneficiaries within the selected Census tracts are sampled annually at rates that depend on age group and Hispanic or non-Hispanic ethnicity. The geographically-clustered nature of the current design was developed to support in-person-only interviewing by ensuring that sampled beneficiaries are clustered into relatively small areas. Under this design, the sampled beneficiaries within each state are not representative of their state as a whole, and 14 states contain no PSUs (and thus no beneficiaries selected for data collection), resulting in a design which supports only national-level estimation. It is not possible to produce representative estimates for states or other sub-national regions.

The 2026 redesign, to be launched starting with the Fall 2026 data collection, will employ a national frame, which covers all 50 states and the District of Columbia. The sample will be stratified by state¹ to provide approximately equal numbers of beneficiary interviews within each state. The redesigned approach will no longer rely on PSUs or SSUs, and all beneficiaries in the United States will be eligible for sampling and recruitment. In place of the PSUs and SSUs, the national frame will be partitioned by state, and within each state, beneficiaries will be selected at rates designed to preserve adequate representation of the under-65 disabled population, beneficiaries 65-79 years of age, and beneficiaries 80 years of age and older. There will no longer be additional stratification based on ethnicity.

While both the current design and the 2026 redesign yield nationally representative samples, the new design will include sampled beneficiaries and completed interviews in each of the 50 states and the District of Columbia. Once fully implemented, the 2026 redesign will allow for state-level and sub-national region-level estimation, while continuing to support national-level estimation for the many domains of interest.

¹ A total of 51 strata that include the 50 states and the District of Columbia.

Phased Implementation Plan

Implementation of the sample redesign will occur incrementally over four years due to the MCBS rotating panel structure:

- **Fall 2026:** The first new panel will be selected and fielded under the redesigned, state-stratified sample. At this time, existing panels will continue to be fielded under the historic geographically clustered design.
- **Fall 2027:** A second panel will be selected and fielded under the redesigned sample. At that point, two panels will operate under the new design and two panels will continue under the legacy design.
- **Fall 2028:** A third panel will be fielded under the new design, and one will remain under the legacy design.
- **Fall 2029:** Full implementation will be achieved when all four rotating panels are operating under the redesigned sample.

The phased redesign will require a forward-looking approach that sustains data quality, continuity, and transparency during the period in which legacy and redesigned panels operate concurrently, while ensuring rigorous statistical integration, operational modernization, and clear communication to stakeholders as new analytic capabilities are introduced.

D. Data Migration and Cloud Hosting

The CMS Office of Information Technology (OIT) cloud system has developed a strategic vision for CMS to become the leading agency in leveraging modern cloud technology. Cloud Computing has afforded CMS the ability to scale up critical applications to meet the agency's program specific goals, while maintaining CMS technical architectural and security standards. CMS desires solutions that implement best practices, employ a Financial Operations (FinOps) culture (promote cost optimization), and deliver business value quickly and continuously using native cloud services, when possible, which provide an abstraction layer over physical and virtual hardware, network and security devices or tools.

CMS is continually conducting Security Impact Assessments (SIA) on new pertinent services identified and desired by the stakeholder community. This is especially true with the expansion and increasing adoption of Artificial Intelligence (AI) capabilities at CMS including machine learning, generative AI, and products derived from AI tools available on the market. CMS continuously reviews industry-provided AI capabilities against CMS security requirements to ensure compliance with CMS and Federal security requirements and the development of standards and guidelines for how AI products and capabilities should be configured to reduce risk and exposure of sensitive CMS data.

E. Questions for Industry

CMS is seeking information from industry regarding the MCBS. Accordingly, please consider the entirety of information provided above when responding to each of the following questions:

1. The MCBS currently relies on Unicom Intelligence and BLAISE 4.8 for administering a complex data collection, with features such as preloaded data, longitudinal logic, and looping. Is there an industry-preferred survey application (i.e., cloud supported, cloud based, or cloud native) that you would recommend for supporting these requirements while operating within the CMS cloud environment? If a change from the existing application is recommended, what level of effort and key considerations would be associated with migrating the survey application?
2. In addition to the survey instrument itself, MCBS requires business operations capabilities to support efficient case management, including tracking and assignment of cases, transitions between community and facility cases, and management of case dispositions. Many contractors support these functions using internally developed systems or commercial off-the-shelf (COTS) tools. What experience and best practices can you describe regarding the use of business intelligence, workflow, or case-management tools to support these functions, including the level of effort and considerations associated with configuring or implementing such solutions in the CMS cloud environment? Please include if this experience is from successful work at CMS or other government agencies and if you used COTS or contractor-developed application.
3. Do you have a best practice for capturing data from paper Explanation of Benefits (EOBs) and other health care receipts as well as information from patient portals in an online, in person, or telephone survey?
4. CMS operates in a dynamic policy environment that can require rapid data collection on emerging or previously unmeasured concepts. In advance of fielding questions through a rapid “pulse” mechanism, what experience and best practices can you describe for quickly developing, testing, and refining new survey questions, particularly where traditional cognitive testing timelines are not feasible?
5. At the time of award, MCBS will be implementing a new sample design, simultaneously fielding cases under both the redesigned sample and the legacy sample (which is being phased out). What best practices, methodologies, or approaches would you recommend for managing this transition period, including strategies for concurrent administration of both sample designs?
6. MCBS faces increasing requirements for data transparency and reporting across the complete data lifecycle—from collection through processing, revision, and dissemination—including comprehensive documentation of changes over time. What best practices do you recommend for developing, maintaining, and sharing metadata in a CMS Cloud-hosted environment? Please address approaches that ensure metadata are structured to support modern analytical and reporting tools, including CMS Cloud-native applications.

7. Does industry recommend one contracting scope mechanism for MCBS to use (Statement of Work (SOW) or Statement of Objectives (SOO) with a Performance Work Statement (PWS))? Please provide an explanation for your recommendation. The MCBS has relied on a SOW in all of its past contracts. This next contract will include innovative enhancements that could reimagine current deliverables.
8. Artificial intelligence capabilities are evolving rapidly, and CMS anticipates that future uses of AI in survey operations and data processing may emerge over the life of the contract. What industry best practices would you recommend for responsibly using AI in survey research and data operations today, and how should CMS plan for potential future AI applications that are not yet defined?
9. The migration of the MCBS system to the CMS Cloud will be completed without a stop in data collection. Keeping in mind the complex system dependencies and handling of sensitive personal data requiring protection (PII and PHI), does industry recommend best practices for tracking progress as well as remedies for schedule or budget slippage during the migration?
10. CMS will be migrating MCBS, currently hosted in an on-premise environment that's managed and owned by a CMS contractor, to a government-owned (CMS Cloud), contractor-operated environment starting with the next contract. What innovative process efficiencies can CMS expect to realize as a consequence of leveraging cloud-native tools during this migration and modernization? And how much time would it take to complete this process?

We appreciate your time and effort in responding to this request for information.

F. Instructions for Submitting RFI Responses to CMS

Email responses are due no later than **12:00 PM EST on April 1, 2026** to **Jessica Foster (Contract Specialist)** at Jessica.Foster@cms.hhs.gov and **Jennifer Davis (Contracting Officer)** at Jennifer.Davis@cms.hhs.gov in the following format:

- Microsoft Word document with page size 8.5 by 11 inches, with a minimum of 1" margins.
- Times New Roman or Aptos Font Size 12
- Submissions shall not exceed **10 pages**. The Cover Page is not included in the total page count.

G. Protection of Privileged or Confidential Commercial or Financial Information

The Freedom of Information Act (FOIA), [5 U.S.C. § 552](#), provides individuals with a right to access certain records in the possession of the Federal government, subject to certain exemptions. Exemption 4 of the FOIA permits information to be withheld that is in the nature of "trade secrets and commercial or financial information obtained from a person and privileged or confidential." The term "person" refers to a wide range of entities, including corporations, banks, state governments, agencies of foreign

governments, and Native American tribes or nations, who provide information to the government. Information that does not qualify withholding under the Exemption will be disclosed.

CMS may invoke [Executive Order 12600](#) in accordance with FOIA Regulation [45 C.F.R. § 5.42](#). Information provided by entities to CMS in response to this RFI may be subject to FOIA disclosure requirements unless protected under Exemption 4 of the FOIA itself. Should entities wish to protect any trade secret or confidential information under Exemption 4, they should mark their submission accordingly citing applicable redactions and non-disclosure rationale. Pre-disclosure redactions and non-disclosure rationale shall be sent via e-mail to the requesting contracting officer with a copy to the CMS FOIA Office at 12600@cms.hhs.gov.

If an entity's submission is not marked and CMS receives a FOIA request related to this RFI, the CMS Contracting Officer will notify the entity. The entity will have 10 working days to object to the release of any part of their information submitted to CMS citing relevant case law. Final determination on the applicability of Exemption 4 of the FOIA are made on a case-by-case basis by [CMS](#).

The statutory language of the FOIA is codified at 5 U.S.C. § 552(b). Additional information and resources, including the 9 FOIA Exemptions and HHS FOIA Regulation, and examples, can be found on the [HHS FOIA Website](#).

H. Other Information:

1. Proprietary Information and Disclaimers: All transmitted information marked proprietary shall be treated as such. Therefore, respondents should identify any proprietary information in its RFI response. Information submitted in response to this RFI will be used at the discretion of the Government. Proprietary materials will neither be distributed, nor discussed with any other organization or business. Further, the information submitted will remain confidential insofar as permitted by law, including the Freedom of Information and Privacy Acts. CMS reserves the right to utilize any non-proprietary technical information in the anticipated SOW or solicitation.
2. Respondents should be aware that this RFI is for market research purposes only and any responses submitted do not constitute a commitment by CMS to treat any respondent more or less favorably in any anticipated forthcoming solicitation and/or ultimate award.
3. Responses to the RFI are not offers and cannot be accepted by CMS to form a binding contract. CMS does not intend to award a contract on the basis of this RFI, or to otherwise pay for the information solicited. No reimbursement for costs will be made associated with providing information in response to this RFI or follow-up information request.
4. Responses to the RFI will not be returned and will not be accepted after the due date. All communications shall be by email.

5. Respondents will not be notified of the results of the review of the responses. Information received will be considered solely to make informed decisions regarding a potential procurement.

On behalf of the Centers for Medicare & Medicaid Services, thank you for your interest in this Request for Information.